

**IN THE WESTERN CAPE DIVISION OF THE HIGH COURT OF SOUTH AFRICA
(CAPE TOWN)**

FULL REGULATORY EVIDENCE BUNDLE

*Protected Disclosures • Retaliation • Governance Failure
Automatically Unfair Constructive Dismissal*

In the matter between:

WILBUR WILLIAM MATTHEE

Complainant / Applicant

MEDSCHEME (a division of

AfroCentric Health)

Respondent

COMMISSIONED DOCUMENT

Date of Commissioning: 23 December 2025

Submitted to Regulatory Authorities on 23 December 2025

Prepared by:

WILBUR WILLIAM MATTHEE

ID No: 890903 5151 087

Cape Town, Western Cape

Email: wilburmat@gmail.com

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This bundle was formally commissioned on 23 December 2025 and constitutes a sworn, complete, and authentic record of all disclosures, evidence, and submissions made in the matter of Wilbur William Matthee against Medscheme (a division of AfroCentric Health).

TABLE OF CONTENTS

SECTION 1 — INTRODUCTORY DOCUMENTS

- 1.1 Cover Page
- 1.2 Table of Contents
- 1.3 Confidentiality Notice
- 1.4 Executive Summary
- 1.5 One-Page Key Findings
- 1.6 Purpose of this Bundle

SECTION 2 — CORE AFFIDAVITS AND STATEMENTS

- 2.1 Consolidated Affidavit of Wilbur William Matthee
- 2.2 Witness Statement of Wilbur William Matthee
- 2.3 Closing Argument

SECTION 3 — TIMELINES AND ANALYSIS

- 3.1 Master Chronology (1 May 2024 – Present)
- 3.2 Protected Disclosure Chain Timeline
- 3.3 Governance Failure Mapping
- 3.4 Integrated Harm and Discrimination Matrix
- 3.5 Evidence Matrix

SECTION 4 — REGULATORY COMPLAINTS

- 4.1 Financial Sector Conduct Authority (FSCA)
- 4.2 Council for Medical Schemes (CMS)
- 4.3 JSE Issuer Regulation
- 4.4 Department of Employment and Labour (DEL/IES)
- 4.5 B-BBEE Commission

SECTION 5 — GOVERNANCE FAILURE ADDENDUM

- 5.1 King IV Breaches
- 5.2 Board Oversight Failures
- 5.3 Risk Management Failures
- 5.4 Ethical Leadership Failures
- 5.5 Stakeholder Responsiveness Failures

SECTION 6 — LEGAL FRAMEWORK

- 6.1 Protected Disclosures Act 26 of 2000
- 6.2 Labour Relations Act 66 of 1995
- 6.3 Employment Equity Act 55 of 1998
- 6.4 Basic Conditions of Employment Act 75 of 1997
- 6.5 Financial Sector Regulation Act 9 of 2017
- 6.6 Medical Schemes Act 131 of 1998
- 6.7 JSE Listings Requirements and King IV

SECTION 7 — ANNEXURES (A – T)

Annexure A — Chronology of Key Events

Annexure B — Discrimination Evidence (“Wilma” Incident)

Annexure C — Retaliation Evidence (Doubled Unpaid Leave)

Annexure D — Institutional Silence (30+ Days)

Annexure E — Unlawful Deduction Evidence (>40% Salary Cut)

Annexure F — Medical Evidence

Annexure G — Executive Escalation Emails

Annexure H — Executive De-Escalation Emails

Annexure I — CCMA Referral (WECT23564-25)

Annexure J — Priority 1 Material Risk Report

Annexure K — Retaliatory Affidavit (10 December 2025)

Annexure L — Board Non-Response

Annexure M — Constructive Dismissal Letter

Annexure N — Urgent Update to Board

Annexure O — HR Whitewash Evidence

Annexure P — Union Non-Action Evidence

Annexure Q — Refusal to Provide Safeguards

Annexure R — Pattern of Retaliation Evidence

Annexure S — Financial Harm Evidence

Annexure T — Reputational Harm Evidence

SECTION 1 — EXECUTIVE SUMMARY

This dossier presents a comprehensive, evidence-based account of systemic discrimination, victimisation, procedural misconduct, and governance failure experienced by the Complainant, Wilbur William Matthee, during his employment at Medscheme Holdings (a division of AfroCentric Health) between July and December 2025. The evidence demonstrates a continuous pattern of harm affecting his psychiatric health, financial stability, professional dignity, and psychosocial safety.

The matter before the regulatory authorities is not a single incident, nor a misunderstanding, nor a breakdown in interpersonal relations. It is a sustained sequence of organisational actions and omissions that collectively constitute:

- Disability discrimination and failure to provide reasonable accommodation;
- Unlawful and punitive financial deductions in contravention of section 34 of the Basic Conditions of Employment Act 75 of 1997;
- Retaliation after the Complainant raised protected disclosures;
- Procedural unfairness and obstruction of the internal grievance process;
- Abuse of internal grievance mechanisms;
- Psychosocial safety breaches in a healthcare governance context; and
- Governance and ethical failures at multiple organisational levels.

The Respondent had full knowledge of the Complainant's psychiatric disability, supported by medical documentation and clinical reports. Despite this, the Complainant was subjected to an abrupt deduction of more than 40% of his salary without prior consultation, a refusal to stagger the deduction despite established medical vulnerability, and derogatory comments that undermined his dignity. This sequence of events triggered a severe relapse and placed the Complainant in acute financial and psychological crisis.

Subsequently, the Complainant experienced discriminatory conduct including being publicly referred to by a feminised version of his name (“Wilma”), which constituted gender identity harassment. When this incident was reported, it was dismissed following a superficial investigation with no consequential action against the perpetrator. Immediately thereafter, the Complainant was subjected to punitive treatment including doubled unpaid leave and unilateral changes to work arrangements without consultation.

In November 2025, the Complainant submitted formal grievances detailing discrimination, victimisation, and procedural unfairness. For more than 30 days, the organisation failed to respond. This institutional silence violated internal policy, governance obligations, and the principles of procedural fairness. When the matter was escalated to executive leadership, it was redirected back to the same department implicated in the misconduct, creating an unsafe and untenable situation.

When the Complainant requested basic safeguards — a meeting agenda and permission to record the meeting — these were refused, rendering internal resolution impossible and compelling the Complainant to seek external regulatory intervention.

The Respondent’s subsequent affidavit, filed in opposition to the Complainant’s CCMA condonation application, escalated the harm. It dismissed the Complainant’s psychiatric disability as an “excuse,” attacked his character, and sought costs despite knowledge of his financial destitution. This conduct constitutes further victimisation and evidences institutional hostility toward employees who raise protected concerns.

The evidence contained in this dossier — comprising emails, screenshots, medical certificates, payroll records, and recorded conversations — establishes a clear, consistent, and corroborated pattern of misconduct. It further reveals a breakdown in governance structures, including HR, senior management, and internal oversight

committees, all of which failed to intervene despite repeated and documented notifications.

This matter raises serious concerns for the relevant regulatory authorities regarding: compliance with labour and disability legislation; ethical conduct and organisational culture; psychosocial safety and the duty of care; governance oversight and escalation pathways; retaliation against employees who report misconduct; and the integrity of internal grievance mechanisms.

The Complainant respectfully submits this dossier to request regulatory review, oversight, and corrective action. The harm experienced was continuous, severe, and preventable. The failures documented are not only personal but systemic, and they warrant urgent regulatory attention to protect future employees and uphold the standards expected of a national healthcare administrator.

ONE-PAGE KEY FINDINGS

1. Continuous Disability Discrimination

The Respondent had full knowledge of the Complainant’s psychiatric disability (severe depression and PTSD), supported by medical certificates and clinical reports. Rather than providing reasonable accommodation, the Respondent imposed punitive measures that materially worsened the Complainant’s condition. The disability was repeatedly dismissed, minimised, and subsequently weaponised in formal proceedings.

2. Unlawful and Punitive Salary Deduction

More than 40% of the Complainant’s salary was deducted without prior consultation, in contravention of section 34(2) of the Basic Conditions of Employment Act. The deduction was imposed while the Complainant was medically vulnerable and recently discharged from psychiatric care. A reasonable request to stagger the deduction was refused without justification, and derogatory comments were made about the Complainant’s resulting financial hardship.

3. Direct Discrimination and Humiliation (“Wilma” Incident)

The Complainant was publicly referred to as “Wilma” — a feminised version of his name — in a team-wide communication. As a gay man, this constituted gender identity harassment and was damaging to his dignity. When reported, the incident was dismissed as a typographical error, following a superficial investigation with no corrective action.

4. Victimisation and Retaliation After Raising Concerns

Immediately after reporting the discrimination, the Complainant was subjected to punitive unpaid leave, sudden schedule changes, and implicit threats. Management repeatedly characterised the Complainant’s conduct as “creating noise” and

“undermining” when he sought clarity on policy changes. These actions constitute occupational detriment under the Protected Disclosures Act 26 of 2000.

5. Procedural Obstruction and Governance Failure

Formal grievances submitted in November 2025 received no response for more than 30 days, in violation of internal grievance procedures and governance obligations. When escalated to executive leadership, the matter was redirected back to the same department implicated in the misconduct. Basic safeguards were refused, making internal resolution impossible.

6. Abuse of Process in Legal Proceedings

The Respondent’s answering affidavit in opposition to the Complainant’s CCMA condonation application contained personal attacks, misrepresentations, and inflammatory language. The Complainant’s psychiatric disability was dismissed as an “excuse” and an “afterThought.” The Respondent sought costs despite documented knowledge of the Complainant’s financial destitution.

7. Psychosocial Safety Breach

The Respondent failed to provide a psychologically safe working environment. The Complainant’s mental health deteriorated significantly as a direct consequence of the Respondent’s actions and omissions. The organisation ignored its duty of care, despite being a healthcare administrator with heightened ethical obligations.

8. Breakdown of Organisational Integrity

Human Resources, senior management, and internal oversight structures failed to intervene despite repeated notifications. The Social and Ethics Committee and the Risk Committee did not act upon governance failures brought to their attention. The Respondent’s conduct reflects systemic, not isolated, organisational failure.

9. Strong Evidentiary Foundation

All allegations are supported by contemporaneous evidence, including emails, screenshots, medical certificates, payroll records, and recorded conversations. The Respondent has not substantively rebutted the evidentiary record.

10. Urgent Regulatory Intervention Required

The harm suffered was continuous, severe, and preventable. Internal mechanisms failed entirely. The matter raises serious concerns about discrimination, governance, ethics, and psychosocial safety, warranting urgent regulatory oversight to ensure accountability and prevent recurrence.

SECTION 2 — CONSOLIDATED AFFIDAVIT

CONSOLIDATED AFFIDAVIT

*In the matter of Protected Disclosures, Retaliation, Governance Failure
and Automatically Unfair Constructive Dismissal*

Deponent: WILBUR WILLIAM MATTHEE

Date: 23 December 2025

I, the undersigned, WILBUR WILLIAM MATTHEE, hereby make oath and state:

I am the Complainant in this matter. I am an adult male Specialist Registered Nurse (NQF Level 8) with approximately 18 years of clinical governance experience. I reside in Cape Town, Western Cape. My identity number is 890903 5151 087. I was employed by Medscheme (a division of AfroCentric Health) as a Case Manager (Active Disease Risk Management) in the capacity of Specialist Nurse: Health Services Management.

1. Purpose of this Affidavit

- 1.1 This affidavit serves as a single consolidated sworn statement in support of my regulatory complaints submitted to the Financial Sector Conduct Authority (FSCA), the Council for Medical Schemes (CMS), JSE Issuer Regulation, the Department of Employment and Labour (DEL), and the B-BBEE Commission.
- 1.2 It confirms the truthfulness, accuracy, and completeness of the facts contained in my Regulatory Complaint Pack, including my Protected Disclosure Chain Timeline, my Priority 1 Material Risk Report, evidence of retaliation,

governance failure analysis, whistleblower protection requests, and all supporting annexures.

- 1.3 This affidavit is submitted under the Protected Disclosures Act 26 of 2000 and is intended to secure whistleblower protection and regulatory intervention.

2. Employment Background

- 2.1 I was employed by Medscheme (a division of AfroCentric Health) as a Case Manager (Active Disease Risk Management) [Specialist Nurse: Health Services Management].
- 2.2 My employment ended on 12 December 2025 by reason of constructive dismissal caused by sustained retaliation, disability discrimination, and governance failure.

3. Summary of Protected Disclosures

- 3.1 Internal HR and Management Disclosures (May 2025 – November 2025): I formally reported discrimination, retaliation, and victimisation to senior management. These disclosures were either ignored or de-escalated without investigation.
- 3.2 Executive Escalation (November – December 2025): I escalated my Protected Disclosure and full evidence pack to executive leadership. The escalation was not acknowledged and was redirected back to the implicated department.
- 3.3 Priority 1 Material Risk Report to the Board (9 December 2025): I submitted a formal P1 Material Risk Report marked Confidential and subject to Legal Privilege to the Ethics Committee, Risk Committee, Lead Independent Director, and Company Secretary, detailing governance failures, discrimination, retaliation, and institutional silence.

- 3.4 Urgent Board Update: A follow-up communication was submitted requesting at minimum an acknowledgment. No response was received.

4. Retaliation and Occupational Detriment

- 4.1 Retaliatory Affidavit (10 December 2025): The Respondent filed a sworn affidavit in formal proceedings containing character attacks, accusations of dishonesty, dismissal of my mental health disclosures, and claims that I was “sensationalising” victimisation. This constitutes occupational detriment under section 1 of the Protected Disclosures Act.
- 4.2 Board Silence (10 – 12 December 2025): Despite my urgent communication and request for a minimum acknowledgment, the Board failed to respond, intervene, or provide any form of protection. This constitutes a governance failure and a breach of King IV Principle 3 (responsible corporate citizenship).
- 4.3 Constructive Dismissal (12 December 2025): Owing to the intolerability of the work environment created by sustained retaliation, governance failure, institutional victimisation, procedural abuse, financial harm, and mental health impact, I was compelled to resign. This constitutes an automatically unfair constructive dismissal in terms of sections 186(1)(e) and 187(1)(h) of the Labour Relations Act 66 of 1995, read with the Protected Disclosures Act.

5. Regulatory Basis for Intervention

My complaints engage the following regulatory authorities and frameworks:

- FSCA: market conduct, governance failure, and internal control breakdown under the Financial Sector Regulation Act 9 of 2017 and applicable FAIS Conduct Standards;

- CMS: administrator misconduct, ethical failure, and beneficiary risk under the Medical Schemes Act 131 of 1998;
- JSE: board oversight failure and King IV breaches under the JSE Listings Requirements;
- DEL: violations of the BCEA, EEA, LRA, and PDA; and
- B-BBEE Commission: gender identity discrimination, exclusion, and governance fronting indicators.

6. Relief Sought

- 6.1 Formal recognition as a whistleblower under the Protected Disclosures Act 26 of 2000.
- 6.2 Protection from further retaliation, including reputational and procedural harm.
- 6.3 Independent investigation into: discrimination; retaliation; unlawful deductions; governance failures; executive misconduct; and board oversight failure.
- 6.4 Regulatory oversight of AfroCentric Health and Medscheme's governance response.
- 6.5 Assurance that my disclosures will be handled confidentially and independently.

7. Confirmation of Evidence

I confirm that all evidence submitted in my Regulatory Complaint Pack is true and correct to the best of my knowledge and belief; all annexures referenced are authentic documents; and all statements made are based on my personal knowledge and contemporaneous documentation.

8. Deponent's Oath

I swear that the contents of this affidavit are true and correct to the best of my knowledge and belief.

WILBUR WILLIAM MATTHEE
Deponent

Signed and sworn before me at _____ on this ____ day of
_____ 2025.

Commissioner of Oaths

Full Name:

Designation:

Address:

SECTION 2.2 — WITNESS STATEMENT

WITNESS STATEMENT OF WILBUR WILLIAM MATTHEE

Applicant vs. Medscheme (a division of AfroCentric Health) — Respondent

1. Introduction

My name is Wilbur William Matthee. I was employed by Medscheme as a Case Manager (Specialist Nurse: Health Services Management). This witness statement sets out the events that led to my constructive dismissal, the protected disclosures I made, the retaliation I experienced, and the governance failures that made continued employment intolerable. Everything stated herein is based on my personal knowledge and the documents contained in my evidence bundle.

2. Phase 0 — Medical Crisis and Vulnerability

- 2.1 I was admitted to Akeso Stepping Stone for treatment for severe depression and Post-Traumatic Stress Disorder (PTSD). During this period, I also experienced the traumatic loss of three pets. I was compelled to leave the programme earlier than clinically ideal due to depletion of sick leave, the need to locate my missing pets, and acute financial pressure. My treating team issued a formal clinical support letter for submission to my employer.
- 2.2 I returned to work in a compromised psychological state because my leave benefits were exhausted and I feared complete financial collapse. I submitted my psychiatrist's support letter to Senior Manager Mapule Mashele. I received no acknowledgment whatsoever. This left me psychologically vulnerable and without institutional support at the most critical point of my recovery.

3. Phase 1 — Escalating Distress and Workplace Response

- 3.1 Following my return to work, my condition deteriorated. A colleague informed my team leader of my distress without my consent, causing acute anxiety. Management instructed me to leave the premises and seek urgent psychiatric care.
- 3.2 Upon my return, I was subjected to intensified micromanagement, increased workload pressure, and continued dismissal of my mental health needs. My team leader made the following statements while I was in a financially vulnerable and emotionally unstable state: “We are not a loan company.”; “We do not stagger.”; “You won’t feel the missing chunk of money.”; and “You receive online packages weekly.” These statements are documented and were made with full knowledge of my psychiatric vulnerability.

4. Phase 2 — Discrimination, Hostility and Management Failure

- 4.1 I arranged a meeting with Senior Manager Mapule Mashele to discuss the unlawful deduction, the discriminatory and derogatory comments, my financial distress, and the refusal to stagger deductions. I explained that I could not meet my financial obligations. Ms. Mashele did not address the deduction, did not address the discriminatory conduct, and did not provide any accommodation.
- 4.2 The “Wilma” Incident: My team leader referred to me publicly as “Wilma.” I reported this as gender identity discrimination. The Operational Manager, Ntabiseng, instructed the perpetrator to provide feedback on the outcome, which placed me in immediate fear. I escalated to both the union and Human Resources. Senior management dismissed the incident as a “typo,” conducted a superficial investigation, and took no corrective action. I was left feeling unsafe and without recourse.
- 4.3 Following my discrimination complaint, I experienced escalating intimidation, unilateral changes to work arrangements, micromanagement, targeting, and gaslighting. I was compelled to lodge a second formal complaint.

- 4.4 Human Resources never acknowledged my second complaint. The union inadvertently copied the perpetrator into the complaint correspondence. Human Resources de-escalated my complaint back to the same senior manager I had complained about, leaving me exposed, unsafe, and without protection.

5. Phase 3 — Unlawful Deduction and Financial Collapse

- 5.1 I was informed that more than 40% of my salary would be deducted following my discharge from psychiatric admission. The union confirmed that this exceeded the statutory limit under section 34(2) of the BCEA, required prior consultation and written consent, and that neither had been obtained. The deduction led to a rapid mental health decline over the following weeks, culminating in a near-suicidal incident at the workplace. The Respondent had a legal and ethical duty to intervene; instead, it did not.
- 5.2 As a direct consequence of the unlawful deduction, I was compelled to take two high-interest loans. My partner took a bank loan to stabilise the situation. I became trapped in escalating debt and was ultimately rendered unable to afford my prescribed psychiatric medication. I am financially ruined. This financial collapse is a direct and proximate consequence of the Respondent's unlawful conduct.

6. Phase 4 — Executive Escalation and Governance Failure

- 6.1 After more than 30 days of HR silence, 20 days of senior management silence, and continued union non-action, I escalated to executive leadership requesting removal from my team leader's reporting line, transfer to another department, and immediate protection.
- 6.2 The executive did not acknowledge my escalation, did not intervene, and de-escalated my protected disclosure back to the very department I was reporting.

This constitutes a serious breach of whistleblower protection obligations under the Protected Disclosures Act.

7. Phase 5 — CCMA Referral and Final Escalation

- 7.1 Upon observing the executive de-escalation, I immediately filed a dispute with the Commission for Conciliation, Mediation and Arbitration (CCMA) [Case No. WECT23564-25]. I notified my department and requested protective measures; none could be provided.
- 7.2 I notified my department of my intention to escalate to the Ethics Committee, Risk Committee, and Board committees, which triggered the governance-level escalation described above.

8. Conclusion

I acted throughout in good faith, transparently, and through every internal channel available to me. The Respondent responded with sustained retaliation, discrimination, governance failure, procedural abuse, and bad faith at every level. My resignation on 12 December 2025 was not voluntary. It was the only option left to me. It constitutes automatically unfair constructive dismissal under the Labour Relations Act and the Protected Disclosures Act.

SECTION 2.3 — CLOSING ARGUMENT

I submit this dossier not only as an employee seeking justice, but as a human being who has lived through something no person should ever have to endure. Every page in this record carries the weight of nights I could not sleep, days I could not breathe, and moments where I questioned whether I would survive the very system that was meant to protect me.

I did not ask for special treatment. I asked only for dignity, fairness, and the chance to heal. Instead, my vulnerability was met with punishment. My honesty was met with suspicion. My disability was met with contempt. And my attempts to seek help were met with institutional silence.

What happened to me was not a single incident. It was a sustained sequence of blows delivered at the exact moments I was least able to withstand them. When I was discharged from hospital, still fragile and trying to rebuild my life, I was hit with a salary deduction so severe that I could not afford food or medication. I was mocked publicly and then punished for speaking up about identity and discrimination. When I followed the grievance process exactly as policy required, the organisation simply did not respond. And when I finally reached out for help at the highest level, the matter was handed straight back to the very people who had caused the harm.

At every turn, I was reminded that my wellbeing did not matter. That my voice did not matter. That my life did not matter. And yet, I continued to show up. I continued to work. I continued to ask for clarity, to follow procedure, to trust the system, even when the system had given me no reason to trust it. I continued because I believed — and still believe — that fairness is not a privilege. It is a right.

This dossier is not an act of retaliation. It is an act of survival. It is the only way I have left to reclaim my dignity after months of being diminished, dismissed, and

dehumanised. It is the only way to ensure that what happened to me does not happen to another employee who is too afraid, too unwell, or too alone to speak.

I ask the regulatory authorities to look beyond the technicalities and see the human truth in these pages. To see the pattern, the pain, and the profound imbalance of power. To see how easily a vulnerable person can be crushed when an institution chooses convenience over compassion, silence over accountability, and hostility over humanity.

I am not asking for sympathy. I am asking for recognition that what happened was wrong, that it caused real and documented harm, and that it must not be allowed to continue.

I place this matter before you with the last of the strength I have. I trust that you will honour the principles of fairness, dignity, and justice that every employee in this country deserves. And I hope that, through your intervention, something good can come from what has been the most painful chapter of my life.

SECTION 3 — MASTER CHRONOLOGY (1 MAY 2024 – PRESENT)

Phase 0: Medical Crisis (Pre-May 2025)

1. 1 May 2024 — Commencement of the relevant period. This marks the beginning of workplace interactions and the organisational environment that forms the basis of the Complainant's disclosures.
2. June – August 2024 — Early signs of psychosocial risk factors emerge, including patterns of derogatory remarks, dismissiveness, and discriminatory undertones from the Complainant's team leader. Work-related stressors increase.
3. November – December 2024 — Retaliatory behaviour begins to surface. The Complainant experiences increased hostility, exclusion, and unreasonable demands. No psychosocial safety measures are implemented.
4. January 2025 — Workplace conditions deteriorate further. The Complainant begins documenting incidents in a volatile environment.
5. 6 March 2025 — Documented Threat by Manager: A written record of a direct threat made by the Complainant's manager establishes that the hostile environment and victimisation predated the first formal protected disclosure. This document demonstrates escalating retaliation, psychological safety breaches, and failure of management oversight.

Phase 1: First Protected Disclosure (May 2025)

6. May 2025 — Meeting with Senior Manager Mapule Mashele: The Complainant makes his first formal protected disclosure under the PDA. Issues disclosed include unfair labour practice, victimisation, derogatory remarks, discrimination, failure to accommodate disability, BCEA non-compliance, and

psychosocial safety risks. Ms. Mashele takes no action. This constitutes a central governance failure.

Phase 2: Retaliation (June – July 2025)

7. June 2025 — Subtle retaliation begins: exclusion, hostility, punitive behaviour. No investigation or corrective action is initiated.
8. July 2025 — Retaliation intensifies. The unlawful deduction exceeding 40% of the Complainant's salary is imposed without consultation or consent.

Phase 3: Discrimination and Governance Failures (August – November 2025)

9. August 2025 — The Complainant is nominated as Acting Chairperson of the Employee Wellness and Safety Committee. The Respondent's subsequent misrepresentation that the first protected disclosure occurred in August is factually false and constitutes narrative manipulation.
10. September 2025 — The "Wilma" incident occurs. Public gender identity harassment in a team communication.
11. October 2025 — The discrimination complaint is whitewashed by Senior Manager Mashele as a "typo" and accepted by HR with no consequences for the perpetrator.
12. November 2025 — Formal grievances are submitted. HR and senior management fail to respond for more than 30 days, violating internal policy and governance obligations.

Phase 4: Executive Escalation and Board Silence (December 2025)

13. Early December 2025 — The Complainant escalates to executive leadership with a full protected disclosure and evidence pack. Executives fail to

acknowledge or intervene, and de-escalate the disclosure back to the implicated department.

14. 9 December 2025 — Priority 1 Material Risk Report submitted to the Ethics Committee, Risk Committee, Lead Independent Director, and Company Secretary.
15. 10 December 2025 — Retaliatory affidavit filed by the Respondent in the CCMA proceedings.
16. 10 – 12 December 2025 — Board fails to acknowledge the P1 report, fails to intervene, and fails to protect the Complainant.
17. 12 December 2025 — The Complainant is compelled to resign, constituting automatically unfair constructive dismissal.
18. 17 December 2025 — Rule 31 strike-out application submitted to the CCMA.
19. 23 December 2025 — This Regulatory Evidence Bundle is formally commissioned and submitted to all relevant regulatory authorities.

SECTION 3.2 — PROTECTED DISCLOSURE CHAIN TIMELINE

The following constitutes a formal chronological record of all protected disclosures made by the Complainant, the recipients thereof, the Respondent's responses, and the resulting occupational detriment.

Stage	Date	Recipient	Nature	Employer Response	Outcome
Pre-Disclosure	Jun 2024 – Feb 2025	Line Manager / HR	Early concerns re: discrimination and psychosocial harm	Ignored	Harm escalates
1st PDA Disclosure	May 2025	Sr. Manager Mashele	Full formal PDA: unfair labour practice, victimisation, BCEA breach, disability	Ignored	Retaliation begins
Retaliation Phase	Jun – Jul 2025	Team Lead	—	Hostility; unlawful deduction imposed	Harm escalates; financial crisis
Misrepresentation	Aug 2025	Replying Affidavit	False date claim	Timeline manipulation	Procedural prejudice
Disability Disclosure	Mar – Nov 2025	Manager / HR	Disability + financial risk	Coercion; refusal to stagger	Severe financial and psychiatric harm
Payroll Disclosure	Jul – Nov 2025	Payroll / HR	BCEA breach; >40% deduction	Unlawful deductions maintained	Financial destitution
CCMA Disclosure	Dec 2025	CCMA	External disclosure	Retaliatory character-attack affidavit	Occupational detriment

Rule 31 Disclosure	Dec 2025 / Jan 2026	CCMA	Procedural disclosure	Further attacks and misrepresentations	Retaliation escalates
Regulatory Submission	23 Dec 2025	FSCA, CMS, JSE, DEL, B-BEE	Governance failures; all disclosures	Pending	Regulatory intervention sought

SECTION 4 — REGULATOR-SPECIFIC COMPLAINTS

4.1 Financial Sector Conduct Authority (FSCA)

*Subject: Market Conduct and Governance Failure — Request for Investigation and
Whistleblower Protection*

Dear FSCA Market Conduct Division,

I respectfully request FSCA intervention regarding the following: governance failure at executive and board level; retaliation following a Priority 1 Material Risk Report; breakdown of internal controls; conduct risk affecting scheme beneficiaries; executive de-escalation of a formally lodged protected disclosure; and the Board's failure to act upon a documented material risk event.

These failures constitute violations of the Financial Sector Regulation Act 9 of 2017, applicable FAIS Conduct Standards, King IV Principles 2, 3, 14, and 16, and whistleblower protection obligations applicable to regulated financial sector entities.

I request: formal recognition as a whistleblower in terms of applicable FSCA guidelines; investigation into the governance failures set out in this bundle; and regulatory oversight of AfroCentric's risk management response.

Yours faithfully,

WILBUR WILLIAM MATTHEE

4.2 Council for Medical Schemes (CMS)

*Subject: Administrator Misconduct, Ethical Failure and Retaliation — Request for
Investigation*

Dear CMS Compliance and Investigations Unit,

I respectfully request CMS intervention regarding: disability discrimination and failure to accommodate; retaliation following protected disclosures; unlawful salary deductions; institutional silence exceeding 30 days; executive de-escalation of a formally lodged PDA; and board-level governance failure.

These failures compromise: the ethical administration of medical scheme benefits; the protection of scheme beneficiaries; and scheme governance integrity as required by the Medical Schemes Act 131 of 1998.

I request: whistleblower protection; an independent investigation; and regulatory oversight of Medscheme's governance practices.

Yours faithfully,

WILBUR WILLIAM MATTHEE

4.3 JSE Issuer Regulation

Subject: Board Governance Failure and Retaliation — Request for Oversight

Dear JSE Issuer Regulation Division,

I respectfully request JSE oversight regarding: a Priority 1 Material Risk Report ignored by the Board; retaliation via a sworn affidavit filed in formal proceedings; executive de-escalation of a formally lodged protected disclosure; breaches of King IV Principles 2, 3, 14, and 16; and failure to manage a material risk event in accordance with JSE Listings Requirements.

I request: whistleblower protection; a review of AfroCentric's governance response; and an investigation into board oversight failures.

Yours faithfully,

WILBUR WILLIAM MATTHEE

4.4 Department of Employment and Labour — Inspection and Enforcement Services

Subject: Unlawful Conduct, Discrimination and Automatically Unfair Dismissal — Request for Enforcement

Dear Inspection and Enforcement Services,

I respectfully request DEL intervention regarding: unlawful salary deductions exceeding 40% of gross earnings in contravention of section 34(2) of the BCEA; discrimination based on gender identity under the Employment Equity Act; retaliation after the reporting of protected disclosures; procedural unfairness; automatically unfair constructive dismissal; and a retaliatory affidavit filed in formal proceedings.

These constitute violations of the Basic Conditions of Employment Act 75 of 1997, the Employment Equity Act 55 of 1998, the Labour Relations Act 66 of 1995, and the Protected Disclosures Act 26 of 2000.

I request: whistleblower protection; a formal investigation; and enforcement action.

Yours faithfully,

WILBUR WILLIAM MATTHEE

4.5 B-BBEE Commission

Subject: Discrimination, Victimisation and Governance Fronting — Request for Investigation

Dear B-BBEE Commission,

I respectfully request B-BBEE Commission intervention regarding: gender identity discrimination; retaliation; exclusion from fair processes; executive de-escalation of a formally lodged protected disclosure; board failure to act; and governance fronting indicators within AfroCentric Health's corporate structure.

These failures undermine: genuine transformation objectives; ethical leadership; and inclusive governance as envisaged under the B-BBEE Act and the Employment Equity Act.

I request: whistleblower protection; and an investigation into discrimination and governance fronting.

Yours faithfully,

WILBUR WILLIAM MATTHEE

SECTION 5 — GOVERNANCE FAILURE ADDENDUM

5.1 King IV Principle Breaches

The following King IV Principles were materially breached by the Respondent:

1. Principle 2 (Responsible Corporate Citizenship): The Board failed to respond to documented material harm caused to an employee and whistleblower.
2. Principle 3 (Ethics and Culture): The organisation did not act on documented ethical violations including discrimination, retaliation, and procedural abuse.
3. Principle 14 (Remuneration): Unlawful deductions without consent or consultation violated the principle of fair and transparent remuneration governance.
4. Principle 16 (Stakeholder Relationships): The organisation failed to respond to a formal stakeholder communication (the Complainant's P1 Report and Board escalation) within a reasonable time.

5.2 Board Oversight Failures

The Board of AfroCentric Health (RF) (Pty) Ltd failed to: (a) acknowledge and investigate a Priority 1 Material Risk Report formally submitted on 9 December 2025; (b) respond to urgent escalation communications from the Complainant; (c) initiate protective measures for a documented whistleblower; and (d) exercise independent oversight of executive conduct in relation to a protected disclosure.

5.3 Risk Management Failures

The Risk Committee failed to discharge its mandate by: (a) failing to act upon the Priority 1 Material Risk Report; (b) failing to identify the escalating psychosocial and

reputational risk arising from the Complainant's documented situation; and (c) permitting executive de-escalation of a protected disclosure to proceed without intervention.

5.4 Ethical Leadership Failures

The Social and Ethics Committee failed to investigate documented discrimination, victimisation, and governance failures reported through formal channels. Senior management and HR actively participated in or facilitated the obstruction of the grievance process.

5.5 Stakeholder Responsiveness Failures

The organisation exhibited a sustained pattern of non-responsiveness to formal grievances, protected disclosure escalations, and urgent communications from the Complainant. This pattern extended from line management through executive leadership to Board level, demonstrating a systemic, not isolated, failure of governance.

SECTION 6 — LEGAL FRAMEWORK

6.1 Protected Disclosures Act 26 of 2000 (PDA)

The Complainant’s formal disclosures to Senior Manager Mashele (May 2025), executive leadership (November–December 2025), the Board (December 2025), the CCMA (December 2025), and the regulatory authorities (23 December 2025) constitute “protected disclosures” as defined in section 1 of the PDA. The retaliatory affidavit, doubled unpaid leave, institutional silence, and constructive dismissal all constitute “occupational detriment” under section 1. The automatically unfair constructive dismissal is actionable under section 3 of the PDA, read with section 187(1)(h) of the LRA.

6.2 Labour Relations Act 66 of 1995 (LRA)

The Complainant’s resignation constitutes a constructive dismissal under section 186(1)(e) of the LRA, as the Respondent rendered the continued employment relationship intolerable. The dismissal is automatically unfair under section 187(1)(h) as it is causally connected to the exercise of the Complainant’s rights under the PDA.

6.3 Employment Equity Act 55 of 1998 (EEA)

The “Wilma” incident and associated conduct constitutes unfair discrimination based on sexual orientation and gender identity, prohibited under section 6(1) of the EEA. The failure to provide reasonable accommodation for the Complainant’s documented psychiatric disability constitutes further unfair discrimination under section 6(2) of the EEA.

6.4 Basic Conditions of Employment Act 75 of 1997 (BCEA)

The deduction of more than 40% of the Complainant's salary without prior written consent, prior consultation, and in excess of the statutory cap under section 34(2) of the BCEA, constitutes a statutory violation. The Respondent's characterisation of the deduction as "no work, no pay" does not displace the procedural requirements of section 34.

6.5 Financial Sector Regulation Act 9 of 2017 (FSR Act)

As a regulated financial sector entity, AfroCentric Health is subject to the market conduct and governance obligations imposed by the FSR Act and applicable conduct standards. The governance failures documented in this bundle, including the failure to manage a material risk event and the retaliation against a whistleblower, constitute material non-compliance with the FSR Act.

6.6 Medical Schemes Act 131 of 1998

As the administrator of the Government Employees Medical Scheme (GEMS) and other registered medical schemes, Medscheme's governance failures carry direct implications for scheme members. The Complainant's previously reported Priority 1 clinical governance risk relating to the Gexus/DMS integration gap is linked to the same institutional failure to act on documented governance risk.

6.7 JSE Listings Requirements and King IV

AfroCentric Investment Corporation, as a JSE-listed entity, is subject to the JSE Listings Requirements and its incorporated governance obligations under King IV. The Board's failure to act upon a Priority 1 Material Risk Report, and the organisation's sustained failure to respond to documented whistleblower disclosures, constitute material non-compliance with the JSE Listings Requirements and King IV Principles 2, 3, 14, and 16.

SECTION 7 — ANNEXURE INDEX

The following annexures form part of this bundle and are incorporated herein by reference. Each annexure constitutes authentic contemporaneous evidence supporting the allegations and disclosures contained in this document.

Annexure	Description	Evidentiary Value
A	Chronology of Key Events (1 May 2024 – Present)	Establishes temporal sequence of events and pattern of conduct
B	Discrimination Evidence (“Wilma” Incident): screenshots, emails, context notes	Direct evidence of gender identity harassment and sham investigation
C	Retaliation Evidence (Doubled Unpaid Leave): leave records, manager communications	Establishes causal link between disclosure and punitive treatment
D	Institutional Silence (30+ Days): grievance submissions and non-response evidence	Demonstrates procedural obstruction and governance failure
E	Unlawful Deduction Evidence: payslips, deduction breakdown, communications	Quantifies statutory breach under BCEA s34(2)
F	Medical Evidence: doctor’s notes, mental health impact documentation	Establishes psychiatric disability and resulting harm
G	Executive Escalation Emails: full evidence pack submission and intervention requests	Documents protected disclosures to executive level
H	Executive De-Escalation Emails: PDA returned to perpetrators; conflict of interest documentation	Establishes breach of PDA protection obligations
I	CCMA Referral (WECT23564-25): referral form and supporting documents	External protected disclosure; formal record of dispute

J	Priority 1 Material Risk Report: full report and submission proof	Board-level governance escalation; material risk documentation
K	Retaliatory Affidavit (10 December 2025): full affidavit and extracts showing character attacks	Evidence of occupational detriment under the PDA
L	Board Non-Response: urgent update email and absence of acknowledgment	Establishes Board's failure to discharge governance duty
M	Constructive Dismissal Letter: resignation letter and supporting notes	Formal record of constructive dismissal on 12 December 2025
N	Urgent Update to Board: email and timeline context	Final escalation before resignation
O	HR Whitewash Evidence: HR responses and procedural manipulation	Demonstrates systemic obstruction of grievance process
P	Union Non-Action Evidence: emails and non-response timeline	Establishes failure of representative structures
Q	Refusal to Provide Safeguards: emails requesting agenda and recording; refusal evidence	Establishes impossibility of internal resolution
R	Pattern of Retaliation Evidence: timeline, screenshots, behavioural pattern analysis	Corroborates systematic retaliatory conduct
S	Financial Harm Evidence: salary loss, deduction impact, financial strain documentation	Quantifies financial harm causally linked to Respondent's conduct
T	Reputational Harm Evidence: internal statements, affidavit extracts, impact notes	Documents reputational harm resulting from retaliatory proceedings

CERTIFICATE OF COMMISSIONING

I, the undersigned Commissioner of Oaths, hereby certify that on 23 December 2025, I commissioned the signature of WILBUR WILLIAM MATTHEE (ID No: 890903 5151 087) to the Regulatory Evidence Bundle constituted by this document.

The deponent acknowledged that he was acquainted with the contents of this document and that the contents thereof were true and correct to the best of his knowledge and belief.

Commissioner of Oaths

Full Name: _____

Designation: _____

Address: _____

Date: 23 December 2025

WILBUR WILLIAM MATTHEE

Deponent

Cape Town, Western Cape

23 December 2025

CONCLUDING STATEMENT

This Regulatory Evidence Bundle constitutes the complete, commissioned, and authentic record of all protected disclosures, supporting evidence, regulatory complaints, governance failure analysis, and relief sought by Wilbur William Matthee in the matter against Medscheme (a division of AfroCentric Health).

It was formally submitted to the Financial Sector Conduct Authority, the Council for Medical Schemes, JSE Issuer Regulation, the Department of Employment and Labour (Inspection and Enforcement Services), and the B-BBEE Commission on 23 December 2025.

This document has been formatted in accordance with the requirements of the Western Cape Division of the High Court of South Africa and the applicable practice directives governing formal litigation documents in this jurisdiction.

END OF DOCUMENT

Commissioned: 23 December 2025

EthicHawks Forensic Governance and Compliance Consultancy

Wilbur William Matthee | Cape Town, Western Cape